

# geha-coverage-policy-lunsumio.pdf

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### Lunsumio, Lunsumio Velo (mosunetuzumab-axgb)

Original Effective Date: 7/1/2023

Revision Date(s): 6/6/2023; 3/14/2024; 4/17/2025; 4/21/2025; 3/16/2026

Review Date: 3/16/2026

Policy type: Medical Necessity

Line of Business: All G.E.H.A Medical Plans.

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability. For patients who achieve a complete response (CR), no further treatment beyond 8 cycles is recommended. For patients who achieve a partial response (PR) or have stable disease (SD), a total of 17 cycles are recommended, unless the patient experiences disease progression or unacceptable toxicity.

### Indication Specific Criteria:

#### Follicular Lymphoma -- Relapsed or Refractory (Grade 1, 2 or 3A)

Lunsumio will be used as monotherapy; AND

The member is 18 years of age or older; AND

The member has not had prior treatment with chimeric antigen receptor T-cell (CART-T) therapy within 30 days before administration; AND

The member has not had an allogeneic hematopoietic stem cell transplant; AND

The member has not had an autologous stem cell transplant within the past 100 days; AND

The member's Eastern Cooperative Oncology Group (EcoG) score of 0 or 2; AND

The member has no history of CNS lymphoma or CNS disorders; AND

The member has no active infection (e.g. viral, bacterial, fungal) such as HIV, active hepatitis B or active hepatitis C; AND

The member has received 2 or more prior lines of therapy that includes anti-CD20 therapy (e.g., rituximab) and an alkylating agent (e.g., cyclophosphamide); AND.

Prescribed by, or in consultation with, an oncologist.

### Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

National Comprehensive Cancer Network® (NCCN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; OR

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United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with the product labeling; OR

Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:

O Clinical Pharmacology (Strong For); OR

O Wolters Kluwer Lexi-Drugs (Level A); OR

Other uses of drugs and biologics may be considered medically necessary if supported as safe and effective according to peer-reviewed articles from one of the following journals:

O American Journal of Medicine; Annals of Internal Medicine; Annals of Oncology; Annals of Surgical Oncology; Biology of Blood and Marrow Transplantation; Blood; Bone Marrow Transplantation; British Journal of Cancer; British Journal of Hematology; British Medical Journal; Cancer; Clinical Cancer Research; Drugs; European Journal of Cancer (formerly the European Journal of Cancer and Clinical Oncology); Gynecologic Oncology; International Journal of Radiation, Oncology, Biology, and Physics; The Journal of the American Medical Association; Journal of Clinical Oncology; Journal of the National Cancer Institute; Journal of the National Comprehensive Cancer Network (NCCN); Journal of Urology; Lancet; Lancet Oncology; Leukemia; The New England Journal of Medicine; Radiation Oncology;

O Meeting abstracts and case reports are excluded from consideration;

Non-standard protocols may be approved based on unique clinical circumstances;

Dose and frequency should be consistent with United States Food and Drug Administration (FDA) labeling, National Comprehensive Cancer Network (NCCN), or indication specific peer-reviewed literature;

In the instance that a request is made for drug(s) that was (were) previously tried (including in the same pharmacologic class or with the same mechanism of action) and such drug(s) was (were) discontinued due to a lack of efficacy the request may be subject to an off-label review for medical necessity unless supported by the NCCN or high quality literature (prospective phase 2 or 3 studies published as full manuscripts in a CMS-supported journal).

## Billing

| Drug Name     | HCPCS Code | Description                         |
|---------------|------------|-------------------------------------|
| Lunsumio      | J9350      | Injection, mosunetuzumab-axgb, 1 mg |
| Lunsumio Velo | J9350      | Injection, mosunetuzumab-axgb, 1 mg |

## References

1. Lunsumio [package insert]. Genentech, South San Francisco, CA. Available at: [https://www.gene.com/download/pdf/lunsumio\\_prescribing.pdf](https://www.gene.com/download/pdf/lunsumio_prescribing.pdf) 2. Lunsumio Velo [package insert]. Genentech, South San Francisco, CA. Available at: [https://www.accessdata.fda.gov/drugsatfda\\_docs/label/2025/761263s006lbl.pdf](https://www.accessdata.fda.gov/drugsatfda_docs/label/2025/761263s006lbl.pdf) 3. Lunsumio. NCCN Drugs and Biologics Compendium. Available at: [https://www.nccn.org/professionals/drug\\_compendium/content/](https://www.nccn.org/professionals/drug_compendium/content/)

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4. Referenced with permission from the NccN Clinical Practice Guidelines in Oncology (NccN Guidelines). National Comprehensive Cancer Network, Inc. 2026. All rights reserved. Accessed March 16, 2026. To view the most recent and complete version of the guideline, go online to.NCCN.org.

5.Budde LE, Sehn LH, Matasar M, SchusterSJ, Assouline S, Giri P, Kuruvilla J, Canales M, Dietrich S, Fay K, Ku M, Nastoupil L, Cheah CY, Wei MC, Yin S, Li CC, Huang H, Kwan A, Penuel E, Bartlett NL. Safety and efficacy of mosunetuzumab, a bispecific antibody, in patients with relapsed or refractory.follicular lymphoma: a single-arm, multicentre, phase 2 study. Lancet Oncol. 2022 Aug;23(8):1055-.1065. doi: 10.1016/S1470-2045(22)00335-7. Epub 2022 Jul 5. PMID: 35803286.

## Disclaimer

Drug Coverage Policies are developed as needed, reviewed and updated at least annually, and are subject to change. Other policies and coverage determination guidelines may apply. Federal and state.regulatory requirements and member specific benefit plan documents, if applicable, must be reviewed prior to utilizing this Drug Coverage Policy. This Drug Coverage Policy is for informational purposes only.and does not constitute medical advice nor dictate how providers should practice medicine. This policy.should not be reproduced, stored in a retrieval system, or altered from its original form without written.permission from OncoHealth, Inc..

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| Date | Updates |
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| 6/6/23 | Policy creation |
| 3/14/24 | Updated Universal Criteria; Updated Indication Specific Criteria; Updated References |
| 4/17/2025 | Annual Review, update to universal criteria. |
| 4/21/2025 | Approved by P&T |
| 3/16/2026 | Policy adopted by G.E.H.A; document updated for universal accessibility.. Annual Review OH
P&T addition of Lunsumio Velo (J9350); approved by OH P&T |
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